S3 3 Assessment

3 評核

Sources / 來源: P1 pp 3–4; P2 slides 11–12; P5; R2 §3 | Open actions / 待辦: G08 G09 G13 G14 G15 G16

3.1 Supervision and clinical gateway / 3.1 督導及臨床准入

The brochure requires continuous supervision by qualified trainers centrally registered with the Programme Management Office and appointed by the Coordinator. Proposed gateway before programme case treatment: verify registration and indemnity, consent and confidentiality training, clinical-site induction, diagnostic records, treatment plan and a practical skills assessment. Record the supervisor's authorisation and permitted tasks. Remote advice must have a named onsite clinician and emergency/referral route.

簡章要求由課程統籌委任並在管理辦公室登記的合資格導師持續督導。建議開始課程病例前核實註冊及彌償保障、知情同意與保密培訓、場地入職導引、診斷紀錄、治療計劃及實務技能評核，並記錄導師授權及准許項目。遙距指導須配合指定現場臨床人員及緊急／轉介途徑。

3.2 Appraisal and case requirements / 3.2 進度評議及病例要求

P1 states a minimum of 20 well-documented supervised cases, three formal written case submissions and presentation of two treated or nearly completed cases. P2 states two enrolled cases and 18 essays. These are unresolved conflicts; no new threshold is imposed here. Confirm whether the two presentations are selected from the three write-ups, which cases count, required case mix, completion/retention rules and arrangements for delayed care. Six-monthly review should record evidence, feedback and an agreed remediation plan (A03).

P1 列至少 20 宗有完整紀錄的受督導病例、三份正式病例報告及兩宗已完成或接近完成病例的匯報；P2 則列兩宗登記病例及 18 篇文章。以上矛盾尚未解決，本稿不擅自訂立新門檻。須確認兩宗匯報是否從三份報告選出、計算準則、病例組合、完成及保持要求與延後治療安排。建議每六個月記錄證據、回饋及協定補救計劃（A03）。

3.3 Examination and marking / 3.3 考試及評分

Documented final components: two-hour MCQ written paper; two treated/almost-completed case presentations; two diagnostic exercises; oral examination. Question count, durations beyond the MCQ, component weightings, pass standard, safety-critical failures, compensation, reassessment, appeals deadlines and examiner appointments are [MISSING]. Proposed quality controls: blueprint every component to outcomes, agree standard setting before examination, calibrate examiners, moderate borderline results, document conflicts and retain secure mark records.

終期考試包括兩小時 MCQ、兩宗已完成／接近完成病例匯報、兩項診斷練習及口試。題數、其餘時限、比重、合格標準、安全關鍵失誤、不及格補償、重考、上訴期限及考官委任均[待補]。建議逐項建立成果藍圖、考前訂標、考官校準、邊緣成績覆核、利益申報及安全保存評分紀錄。

3.4 Award and continuing education / 3.4 授證及持續教育

P1 requires all prescribed attendance and CDE of at least 10 hours per annum, also stating at least 30 hours across 2.5 years. Do not silently replace this with 25 hours; confirm partial-year rules. Award sign-off should reconcile attendance, approved workload, assessments, portfolio, supervision and unresolved conduct concerns. Certificate wording and any MOrth progression claims require institutional and RCSEd confirmation.

P1 要求出席全部指定活動，每年持續牙科教育至少 10 小時，同時列兩年半內合共至少 30 小時。不可自行改成 25 小時；須釐清不足一年的計算方式。授證前應核對出席、獲批學習量、評核、病例、督導及未解決操守事項。證書及任何 MOrth 銜接字句須經機構及 RCSEd 確認。